

1. Administrative & Study Identification

Full Study Title: Long-Term Safety Study of Deucravacitinib Versus Ustekinumab in Participants With Psoriasis (PRAGMATYK)
Short Title/Acronym: PRAGMATYK **Protocol Number:** IM011-1130 **NCT Number:** NCT07116967 **Sponsor Name:** Bristol Myers Squibb (BMS)
Investigational Product: Deucravacitinib (Active Comparator: Ustekinumab) **Phase of Development:** Phase 3 **Trial Status:** Recruiting **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate the long-term safety, specifically measuring the composite cardiovascular adjudicated 3-point major adverse cardiovascular event (MACE) plus coronary revascularization, of deucravacitinib in comparison to ustekinumab. **Secondary Objectives:** To evaluate the long-term safety profile, including adverse events (AEs) and serious adverse events (SAEs), as well as the long-term effectiveness/efficacy of deucravacitinib versus ustekinumab in treating moderate-to-severe plaque psoriasis.

2.2 Background & Rationale Millions of adults struggle with chronic small plaque psoriasis and require long-term systemic therapies. This study aims to better understand the long-term safety of deucravacitinib, an oral FDA-approved medication, compared to ustekinumab, a biologic injection. The trial focuses specifically on patients aged 40 years or older who have moderate-to-severe active plaque psoriasis and possess one or more cardiovascular risk factors, addressing a critical need for safety data regarding cardiovascular outcomes in this specific patient population.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Active Comparator (Ustekinumab) **Blinding:** Open-label **Randomization:** Yes (Participants are randomly assigned to either the deucravacitinib or ustekinumab treatment group in a multi-center setting)

3.2 Planned Enrollment & Duration **Target N\$:** 3,040 participants **Number of Sites:** Approximately 342 locations globally **Estimated Study Start:** 22-Sep-2025 **Estimated Primary Completion:** 16-Jan-2031

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults aged 40 years or older. 2. Confirmed diagnosis of moderate-to-severe plaque psoriasis, and deemed by the investigator to be a candidate for phototherapy or systemic treatment (including ustekinumab). 3. Presence of at least 1 cardiovascular risk factor: * Current cigarette smoker * Diagnosis of hypertension * Diagnosis of hyperlipidemia * Diabetes mellitus type 1 or 2 * Obesity * History of one or more of the following cardiovascular events: Coronary intervention (PCI) or coronary artery bypass grafting (CABG), myocardial infarction (heart attack), cardiac arrest, hospitalization for unstable angina, acute coronary syndrome, stroke, or transient ischemic attack. * Family history of premature coronary heart disease or sudden death in a first-degree male relative younger than 55 years of age or in a first-degree female relative younger than 65 years of age.

4.2 Exclusion Criteria 1. Unstable cardiovascular disease (CVD) defined as a recent clinical cardiovascular event (e.g., unstable angina, rapid atrial fibrillation), or a recent history (within 90 days prior to Day 1) of a cardiovascular event such as myocardial infarction, stroke, or coronary revascularization, or venous thromboembolism (VTE), or a cardiac hospitalization (e.g., heart failure, pacemaker implantation). 2. Evidence of active cancer, or a history of cancer (solid organ or hematologic malignancy including myelodysplastic syndrome) or lymphoproliferative disease within the previous 5 years (exceptions made for resected cutaneous basal cell or squamous cell carcinoma, or carcinoma of cervix in situ that has been treated with no evidence of recurrence).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Random assignment to either oral daily tablets (Deucravacitinib) or subcutaneous injections (Ustekinumab [Trade Name: Stelara; ATC Code: L04AC05]). **Route of Administration:** Oral (Deucravacitinib) and Subcutaneous (Ustekinumab). **Duration of Treatment:** Up to 5 years.

5.2 Concomitant & Prohibited Therapy Permitted: Standard-of-care treatments for managing pre-existing cardiovascular risk factors and co-morbidities. **Prohibited:** Concurrent use of other systemic immunosuppressive therapies, biologic treatments, or phototherapies targeting psoriasis during the trial period.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Up to 4 weeks prior Informed

Consent, Medical History, Fasting Lipid Panel, Liver Function Tests | | **Baseline** | Day 1 | Randomization, First Dose/Injection, Baseline Assessments | | **Interim** | Weeks 12, 26, then every 6 months | Safety Labs, Efficacy Assessment, Cardiovascular Monitoring, Study Drug Dispensation | | **End of Study** | Year 5 (Week 260) | Final Safety Follow-up, Efficacy Evaluation, End of Study Visit |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Composite cardiovascular adjudicated 3-point major adverse cardiovascular event (MACE) plus coronary revascularization. MACE is defined as non-fatal myocardial infarction (MI), nonfatal stroke, and cardiovascular death. **Measurement Method:** Clinical evaluation followed by an independent central Cardiovascular Adjudication Committee over the 5-year timeframe.

7.2 Secondary & Exploratory Endpoints * Number of participants with non-fatal MI. * Number of participants with non-fatal stroke. * Death due to cardiovascular events. * Incidence of Treatment-Emergent Adverse Events (AEs) and Serious Adverse Events (SAEs) over 5 years. * Long-term effectiveness of deucravacitinib versus ustekinumab on plaque psoriasis clearance.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for unwanted or harmful effects throughout the 5-year study via 6-month periodic clinic visits. Routine fasting lipid panels and liver function tests will be conducted. **Serious Adverse Events (SAEs):** Standard protocol collection; reported typically within 24 hours of site awareness. **Data Monitoring Committee (DMC):** An independent central adjudication committee will review and validate all major cardiovascular events (MACE).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy/safety; Safety Set for all participants receiving at least one dose. **Sample Size Justification:** Target enrollment of $N = 3,040$ to provide sufficient statistical power ($\alpha = 0.05$, $\text{Power} = 80\%$) to compare the long-term safety and incidence of MACE between the two interventions. **Handling of Missing Data:** Standard multiple imputation (MI) and time-to-event analysis models (e.g., Cox proportional hazards) for long-term continuous and survival endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Onsite clinical visits are scheduled at Weeks 12, 26, and every 6 months thereafter for the remainder of the 5-year study. **Audit Trail:** eCRF locking and tracking of cardiovascular events will be subjected to robust data quality audits and independent adjudication.

11. Study Identifiers & Governance

Primary ID: IM011-1130 **Posting ID:** 536837331388002292 **Registry Number:** NCT07116967 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** PRAGMATYK Study **Official Regulatory Title:** A Phase 3b/4 Multi-center, Randomized, Open-label, Long-term Safety Study of Deucravacitinib in Comparison to Ustekinumab in Participants With Moderate-to-Severe Plaque Psoriasis

12. Clinical Framework (Psoriasis & Cardiovascular Safety Specific)

Primary Condition: Moderate-to-severe plaque psoriasis (Chronic small plaque psoriasis) with cardiovascular risk **Diagnosis Threshold:** Clinical indication requiring systemic therapy or phototherapy **Medical Methodology:** Long-term Oral vs. Biologic Systemic Therapy **Optimization Target:** Disease clearance and mitigation of cardiovascular risk incidence

13. Study Procedures & Methodology

Management Pathway: Standard clinical management for moderate-to-severe psoriasis paired with rigorous longitudinal cardiovascular event tracking. **Education Protocol (HCP):** Training on protocol-specific reporting of MACE criteria and cardiovascular risk stratification. **Education Protocol (Patient):** Education regarding cardiovascular risk factor management, study drug adherence, and safety reporting over the 5-year period. **Quality Audit Mechanism:** Independent blinded adjudication of all suspected cardiovascular events (MACE + revascularization).

14. Observation & Follow-Up Schedule

Total Duration: Up to 5 years (approx. 260 weeks) **Onsite Visit Cadence:** Week 12, Week 26, and every 6 months thereafter **Remote Monitoring Frequency:** Standard ongoing surveillance between 6-month clinic visits. **Checklist Review Interval:** Every 6 months during scheduled on-site assessments.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					